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期刊论文标题、摘要、结论部分撰写 prompt

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论文标题生成 prompt

You are working as a professor in the field of [sensing technologies 领域] and a master of language editing. I'm preparing to submit my research article to the [Sensors 目标期刊名称]. Based on the Abstract I provided, please generate **3 proper candidate title**. The title should be concise, informative, and engaging, and should incorporate relevant keywords for better discoverability within the field of [sensing technologies 领域].

[Abstract]

大模型：Claude sonnet 4.5

标黄部分换成你的领域，目标期刊，摘要部分。

论文摘要撰写 prompt

#Role:

As an expert in [Medicine], with a comprehensive and in-depth academic background in [Medicine] and extensive experience in academic writing for [Nature Communications].

##Goal:

Please draft an abstract in accordance with the content of the introduction and results sections of my paper, ensuring that it aligns with the theme of my research. The paper will be submitted to the journal [Nature Communications].

##Background:

【把你的论文相关的标题、前言和结果部分全部粘贴到这】

Title: A global analysis of dairy consumption and incident cardiovascular disease

Introduction

Cardiovascular disease (CVD) is the largest contributor to death globally¹. Adopting healthy dietary patterns is one of the cornerstones of primary prevention of CVD. Thereinto, although dairy consumption features in many dietary guidelines, its role in a heart-healthy diet remains highly debated². Dairy products contain various beneficial nutrients, including high biological value protein, milk fat globule phospholipids, and vitamins and minerals that could improve CVD risk factors^{3,4,5,6}, whereas saturated fats⁷ and multiple anabolic hormones might adversely affect the health benefit, such as IGF-18,9. Previous prospective studies linking dairy consumption with CVD outcomes have yielded conflicting results. Some cohort studies reported a protective relationship between dairy consumption and CVD outcomes^{10,11,12,13,14}, whereas others showed no significant associations^{15,16,17,18} or even positive associations^{19,20}. Meta-analyses also yielded inconsistent conclusions on associations of dairy intake with coronary heart disease (CHD) and stroke risk^{21,22}. Notably, heterogeneity between included studies was considerable and the overall quality of the evidence was low to moderate.

Prevailing recommendations advocate low-fat or non-fat dairy over whole-fat dairy²³. However, scientific evidence for this recommendation was scant and inconsistent²¹. Importantly, different subtypes of dairy products may confer divergent health effects after processing. Fermented milk products such as yogurt contain probiotics that can favorably regulate gut microbiome²⁴, whereas cheese is rich in sodium which may elevate blood pressure when consumed in excessive amounts²⁵. Nonetheless, cheese is also a fermented food that can contain vitamin K²⁶, high levels of milk fat globule membrane²⁷, as well as probiotics²⁸. Furthermore, previous epidemiological studies were largely conducted in Western countries, where the consumption level of dairy products especially cheese is high and usually correlated with a higher socioeconomic position^{29,30}. In Asia where strokes are more common than CHD, only a few studies demonstrated an inverse association of dairy consumption with stroke^{10,31}. Overall, evidence from large cohort studies in both Western

and non-Western countries is needed to make global policy recommendations.

To address the above-mentioned gaps in knowledge, we followed 0.9 million individuals from the UK Biobank (UKB) study and the China Kadoorie Biobank (CKB) study to evaluate the associations of dairy product consumption with incident CVD, CHD, and stroke. We also performed an updated systematic review of the literature and meta-analysis of dairy product intake and incident CVD risk which included our findings to address the role of dairy consumption in CVD prevention and improve dietary guidelines.

In this study, we demonstrate that total dairy consumption is inversely associated with the overall risk of CVD and stroke. Higher intake of dairy products is significantly linked to a reduced risk of stroke in the Chinese population, while it is associated with a lower risk of CVD, CHD, and ischemic stroke in the British population. When examining specific dairy subtypes, cheese, and low-fat dairy products emerge as potentially protective and may be recommended for CVD prevention.

Results

Cohort analyses

During a follow-up of 4,190,676 person-years in CKB and 4,736,113 person-years in UKB, we documented 66,132 CVD cases in CKB and 32,822 CVD cases in UKB. In CKB, participants who consumed dairy products more frequently tended to be women, higher-educated, high-income class, urban residents and vitamin and mineral supplements users, have diabetes and family history of CVD, and consume fruits and eggs more frequently (Supplementary Data 1). In UKB, individuals with higher total dairy consumption were more likely to exercise, be more educated, take vitamin and mineral supplements, and consume oily fish and fruits more frequently, whereas they drank alcohol less frequently and had a lower hypertension prevalence (Supplementary Data 2). Characteristics of participants by cheese consumption (the main subtype of dairy in UKB) and milk types in UKB are shown in Supplementary Data 3 and 4.

Compared to non/rare consumers, those who consumed at least 4 times/week of dairy had no significant association with CVD after the multivariable adjustment in CKB (HR 1.00, 95% CI 0.97–1.03, P-trend = 0.470). Regular dairy consumption was related to a 9% higher risk of CHD (HR 1.09, 95% CI 1.05–1.13, P-trend < 0.001) but a 6% lower risk of stroke (HR 0.94, 95% CI 0.91–0.97, P-trend = 0.005), especially hemorrhagic stroke (HR 0.76, 95% CI 0.69–0.83, P-trend < 0.001) (Table 1). Similar associations of CVD, CHD, and stroke were detected for the long-term usual dairy intakes (per 50 g/d increment) (Supplementary Data 5). In UKB, total dairy intake was inversely associated with incident CVD (HR 0.93, 95% CI 0.88–0.98, P-trend = 0.004), CHD (HR 0.93, 95% CI 0.88–0.99, P-trend = 0.014), and ischemic stroke (HR 0.86, 95% CI 0.75–0.99, P-trend = 0.036) (Table 2).

For individual dairy products, cheese (46.49%) and yogurt (35.69%) were the majority (Supplementary Fig. 1a). Cheese consumption was associated with lower CVD and CHD risk. The HRs (95% CIs) comparing the frequency at least 7 times/week of cheese with less than 2

times/week were 0.88 (0.83–0.94) for CVD, 0.88 (0.82–0.94) for CHD, and 0.97 (0.85–1.11) for stroke in the fully adjusted model (Supplementary Data [6](#)), which was similar to the results from 24-h dietary recalls (Supplementary Data [7](#)). For subtypes of cheese, the proportions of each subtype are shown in Supplementary Fig. [1b](#) and we found both hard cheese and fresh cheese (>0.5 serving/d) were associated with a lower risk of CVD and CHD (Supplementary Data [8–10](#)). Considering the fat content of cheese, a protective association with CVD and CHD was found for high-fat cheese (>0.5 serving/d) while low-fat cheese was negatively associated with stroke incidence, especially ischemic stroke (Supplementary Data [11](#) and [12](#)). Milk consumption (>0 to 0.5 serving/d) was associated with a lower risk of hemorrhagic stroke (HR 0.43, 95% CI 0.21–0.87), and yogurt consumption (>0.5 serving/d) was related to decreased ischemic stroke risk (HR 0.86, 95% CI 0.77–0.98), compared with non-consumers (Supplementary Data [13](#) and [14](#)). No significant relationships were detected for ice cream consumption (Supplementary Data [15](#)). Regarding different types of milk, compared with participants who never or rarely drank milk, both semi-skimmed and skimmed milk consumers had decreased CVD risk (semi-skimmed: HR 0.92, 95% CI 0.87–0.98; skimmed: HR 0.91, 95% CI 0.86–0.97) and stroke risk (semi-skimmed: HR 0.80, 95% CI 0.71–0.90; skimmed: HR 0.76, 95% CI 0.66–0.86). Attentionally, the association of whole milk (HR: 0.93, 95% CI: 0.87–1.00) with CVD incidence was marginally inverse (Supplementary Data [16](#)). Results for subgroup analyses in CKB and UKB were shown in Supplementary Data [17–20](#). Notably, the inverse associations between dairy consumption and the risks of CVD and stroke were observed exclusively in men, not women (P -interaction < 0.001), and in individuals with hypertension, but not in those without hypertension (P -interaction < 0.001), in the CKB (Supplementary Data [17](#)). The inverse association of dairy consumption and CVD risk was detected in current smokers but not in non-smokers (P -interaction = 0.007) in UKB (Supplementary Data [18](#)). Moreover, our results did not alter substantially in sensitivity analyses (Supplementary Data [21–24](#)). In hypothetical substitution analyses, no significant associations were found in UKB. In CKB, replacing 50 g/d of eggs with an equivalent amount of dairy products was associated with an 11% higher risk of CVD, a 13% higher risk of CHD, and a 9% higher risk of stroke. In addition, substituting dairy products for fish or soybeans was associated with a 4% increase in CHD risk, whereas replacing red meat or soybeans with dairy products was associated with a 2% or 3% reduction in stroke risk, respectively (Supplementary Fig. [2](#)).

Systematic review and meta-analysis

Overall, 30 publications from 25 prospective cohorts and our results from CKB and UKB were kept in our final meta-analysis (Supplementary Fig. [3](#), Supplementary Data [25](#) and [26](#)). During a range of 5.5 to 30.0 follow-up years, 73,193 CVD cases were documented among 1,288,420 participants from 30 countries or territories around the world in the previous studies (30 studies) (Supplementary Data [27](#)).

In the meta-analysis of previously published studies, a marginal inverse association was identified between total dairy intake and incident cardiovascular disease (CVD) (RR, 0.963; 95% CI, 0.926–1.001; n = 24 risk estimates). When the results from the CKB and UKB studies were incorporated, the 95% CI of the summary RR narrowed to 0.963 (0.932–0.995) (Fig. [1](#)). Each serving/day increment of total dairy products was related to a 2% lower CVD risk (RR 0.98, 95%

CI 0.96–0.99, $P < 0.001$, $n = 17$ risk estimates) (Supplementary Fig. 4). A similar inverse relationship for CVD was also shown in non-linear analysis (P -nonlinear = 0.002, $n = 12$ studies, Supplementary Fig. 5). For subtypes of CVD, the meta-analysis showed dairy consumption had an inverse relationship with total stroke risk (RR 0.94, 95% CI 0.90–0.98, 14 risk estimates, $I^2 = 61.8\%$) but a null association with CHD risk (RR 0.98, 95% CI 0.93–1.02, 19 risk estimates, $I^2 = 70.5\%$, Fig. 1).

For major subtypes of dairy products, high intake of fermented dairy products, especially cheese, had a protective association with CVD risk (RR for fermented dairy 0.96, 95% CI: 0.94–0.98, $n = 24$ risk estimates; RR for cheese 0.94, 95% CI: 0.91–0.97, $n = 20$ risk estimates), but not yogurt (RR 0.99, 95% CI 0.93–1.06, $n = 14$ risk estimates) or milk (RR 1.00, 95% CI 0.97–1.04, $n = 21$ risk estimates) (Fig. 2 and Supplementary Fig. 6). Cheese intake was also associated with a decreased risk of CHD and stroke (Supplementary Figs. 7 and 8). Considering the content of fat, consumption of low-fat dairy products was significantly related to lower total CVD (RR: 0.96, 95% CI: 0.92–0.99, $n = 20$ risk estimates) and stroke risk (RR: 0.90, 95% CI: 0.84–0.98, $n = 9$ risk estimates) (Fig. 3 and Supplementary Fig. 9). Consumption of high-fat dairy products (including high-fat milk, high-fat yogurt, high-fat cheese, and cream or butter) was not associated with CVD risk (RR: 0.97, 95% CI: 0.93–1.01, $n = 21$ risk estimates) but inversely associated with CHD risk (RR: 0.96, 95% CI: 0.93–0.99, $n = 14$ risk estimates) (Fig. 3 and Supplementary Fig. 10). For subtypes of stroke, milk consumption was related to a higher risk of hemorrhagic stroke (RR 1.08, 95% CI 1.01–1.17, $n = 5$ risk estimates) and a decreased ischemic stroke risk was detected for total dairy (RR 0.92, 95% CI 0.86–0.99, $n = 7$ risk estimates) and cheese consumption (RR 0.91, 95% CI 0.85–0.97, $n = 4$ risk estimates) (Supplementary Figs. 11–14).

For total dairy consumption, we observed considerable heterogeneity across the studies ($I^2 = 66.1\%$) but did not find any publication bias (Fig. 1 and Supplementary Figs. 15–19). No significant heterogeneity was found in the predefined subgroup (sex, follow-up duration, region, Newcastle-Ottawa Scale score, etc.) meta-regressions (Supplementary Data 28), indicating the source of heterogeneity mainly comes from subtypes of dairy. No single study disproportionately caused the heterogeneity (Supplementary Fig. 20). Results of influence analysis for subtypes of dairy and subtypes of CVD are shown in Supplementary Figs. 21–30. If no significant heterogeneity was found across the studies for specific meta-analyses, we also conducted a fixed effects model to calculate summary HRs and 95% CIs which showed similar results (Supplementary Data 29). Results of the GRADE confidence in the estimates of associations are presented in Supplementary Data 30, indicating overall evidence of very low to moderate quality.

##Skills:

###Framework

- ✓ Start by highlighting the research background and existing problems/controversies.
- ✓ Clearly state the research objective or hypothesis.
- ✓ Present the results in order of importance, emphasizing findings that differ from

expectations or offer new insights.

- ✓ Use precise percentages or numerical changes when describing the conclusions.
- ✓ End with a summary conclusion, highlighting the significance or potential applications of the study.

###Language Style

- ✓ Use precise scientific terminology and avoid colloquial expressions.
- ✓ Ensure data presentation is accurate, retaining decimal points.
- ✓ Maintain logical flow with appropriate transition words (e.g., however, therefore, furthermore) and avoid abrupt topic shifts.
- ✓ Maintain objectivity and neutrality, using descriptive language without subjective evaluations.
- ✓ Keep sentences concise and avoid redundancy.
- ✓ Engagement: Reflect the value of the research in the Background section, emphasize key findings in the Results section, and highlight the significance of the research in the Conclusions section.

##Example:

【提供 2 个目标期刊上的标题+摘要，作为 example，让 Claude 学习】

< Example 1>

Small molecule APOL1 inhibitors as a precision medicine approach for APOL1-mediated kidney disease

Chronic kidney disease affects ~10% of people worldwide and there are no disease modifying therapeutics that address the underlying cause of any form of kidney disease. Genome wide association studies have identified the G1 and G2 variants in the *apolipoprotein L1 (APOL1)* gene as major contributors to a subtype of proteinuric kidney disease now referred to as APOL1-mediated kidney disease (AMKD). We hypothesized that inhibition of APOL1 could have therapeutic potential for this genetically-defined form of kidney disease. Here we describe the development of preclinical assays and the discovery of potent and specific APOL1 inhibitors with drug-like properties. We provide evidence that APOL1 channel activity drives podocyte injury and that inhibition of this activity stops APOL1-mediated cell death and kidney damage in a transgenic mouse model. These preclinical data, combined with clinical data from our previously published phase 2 proof-of-concept study, support the potential of APOL1 channel inhibition for the treatment of AMDK.

< Example 2>

Homeostatic microglia initially seed and activated microglia later reshape amyloid plaques in Alzheimer's Disease

The role of microglia in the amyloid cascade of Alzheimer's disease (AD) is debated due to conflicting findings. Using a genetic and a pharmacological approach we demonstrate that depletion of microglia before amyloid- β (A β) plaque deposition, leads to a reduction in plaque numbers and neuritic dystrophy, confirming their role in plaque initiation. Transplanting human microglia restores A β plaque formation. While microglia depletion reduces insoluble A β levels, soluble A β concentrations stay consistent, challenging the view

that microglia clear A β . In later stages, microglial depletion decreases plaque compaction and increases neuritic dystrophy, suggesting a protective role. Human microglia with the *TREM2*(R47H/R47H) mutation exacerbate plaque pathology, emphasizing the importance of non-reactive microglia in the initiation of the amyloid cascade. Adaptive immune depletion (*Rag2*^{-/-}) does not affect microglia's impact on plaque formation. These findings clarify conflicting reports, identifying microglia as key drivers of amyloid pathology, and raise questions about optimal therapeutic strategies for AD.

##Constraints:

1. When a term first appears, provide both the full name and abbreviation, formatted as "full name (abbreviation)"; ensure consistency in term usage throughout the abstract.
2. The drafted abstract must align with the content of the ##Background section and cannot deviate from it.
3. Based on the Results section, all key findings must be included in the abstract, with no omissions.

##Workflows:

1. Study the abstract writing requirements and style outlined in the ##Skills and ##Example sections.
2. Review the ##Background section on the research and results content.
3. Draft an abstract that matches the target journal's style and can capture the interest of reviewers and readers.
4. Think step by step to ensure that all requirements are met.

使用说明：

1 大模型：Claude 4.5；

2 标黄的部分换成你自己相关的：

- ✓ Background 部分，换成你的标题、前言、结果部分内容；
- ✓ Example 部分，换成目标期刊上的标题+摘要；
- ✓ 其他领域，目标期刊，也换成你自己需要的。

论文 Conclusions 结论撰写提示词 prompt

-By 迪娜学姐 ©版权所有

You are a professor in the field of [Nursing 目标领域], with 20 years of research experience and 100 top journal publications. The following paper is my manuscript, which I am about to submit to [BMC Open 目标期刊]. Please help me write a one-paragraph **Conclusion** section based on my title, abstract, and results section.

Requirements:

Write in English, maintaining consistency with the original writing style of my paper while adhering to an academically neutral and objective tone.

Example:

【提供目标期刊上的一篇相似 Conclusion 写法的文章的标题、摘要和 Conclusion 部分，供 AI 学习模仿】

Title: Current situation and support need for non-cancer patients' admission to inpatient hospices/palliative care units in Japan: a nationwide multicenter survey

Abstract:

Background: Specialist palliative care for non-cancer patients is important; however, access to inpatient hospices/palliative care units (PCUs) for non-cancer patients in Japan may be insufficient. We aimed to explore the current situation, the support needs to accept admission of non-cancer patients, and the willingness to accept admission of non-cancer patients to inpatient hospices/PCUs in Japan.

Methods: We conducted a nationwide multicenter anonymous questionnaire survey to inpatient hospices/PCUs in Japan in January 2022. We recruited potential participants from 381 PCUs belonging to *Hospice Palliative Care Japan* (HPCJ).

Results: A total of 264 of 381 facilities responded to the survey (response rate: 69.3%) and 75.0% replied that it was "very necessary" or "necessary" to provide health care coverage of hospitalization costs of non-cancer patients to the same level as cancer patients in inpatient hospices/PCUs. Furthermore, 59.1% replied that they would be "willing" or "somewhat willing" to admit non-cancer patients under the assumption that hospitalization costs covered by health care insurance. In addition, 15.2% of facilities had admitted non-cancer patients. A need for clarification of admission criteria for chronic heart failure (CHF) ($r_s = -0.166$, $P=0.008$), chronic respiratory failure ($r_s = -0.146$, $P=0.019$), chronic hepatic failure ($r_s = -0.161$, $P=0.010$), and chronic renal failure (CRF) with dialysis ($r_s = -0.151$, $P=0.017$); the need for an education and training system for chronic respiratory failure ($r_s = -0.132$, $P=0.034$); and advice from experts in the hospital for chronic respiratory failure ($r_s = -0.156$, $P=0.013$) were significantly negatively associated with willingness to accept the admission of non-cancer patients.

Conclusions: A total of 15.2% of facilities had admitted non-cancer patients under the

current situation, and 59.1% of the facilities were willing to accept the admission of non-cancer patients under the assumption that hospitalization costs would be covered by health care insurance to the same level as cancer patients. Our study highlighted the importance of the establishment of a health insurance system in which appropriate palliative care is available regardless of disease, the definition of admission criteria, and the establishment of a systematic educational program.

Conclusions:

Our study indicated that non-cancer patients' admission to inpatient hospice/PCUs was not standard in Japan due to a lack of health care insurance coverage, clarity on admission criteria, and education on the palliative care of non-cancer patients. Therefore, we propose the establishment of a health insurance system in which appropriate palliative care is available regardless of disease, clarification of the admission criteria, and establishment of a systematic education program.

Workflow:

1. Study the writing style and content scope of the ## Example section's *Conclusions*.
2. Follow the writing norms specified in the ## Requirements section.
3. Prompt the user to input the title, abstract, and results section of their paper, then draft the **Conclusion** section accordingly.

使用说明：

工具选择：Claude sonnet 4.5

- 1 黄色高亮换成自己的；
- 2 先全部输入提示词，等待 Claude 回复；
- 3 根据回复提示，输入你的论文相关的标题、摘要和 Results 部分，Claude 会根据你的论文内容输出匹配的 Conclusions 部分。
- 4 如果你的目标期刊论文的 Conclusions 部分不是一段式的，把提示词中的绿色高亮部分删掉即可，它会模仿你给的目标期刊的风格来写。